جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : احلام حامد محمد سيد
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** الهرم
رقم البطاقة : 29311162102685 **رقم التليفون :** 01252014100 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 16-11-1993 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: احلام حامد محمد سيد
National ID: 29311162102685

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٩٣١١١٦٢١٠٢٦٨٥ ٢٠٢٥/٠٤

بدون عمل

انثى مسلمة متزوجة

مجدى عطيه ابراهيم محمد

١ البطاقة سارية حتى ٢٠٣٢/٠٤/١٦



جمهورية مصر العربية

بطاقة تحقيق الشخصية



إحلام

حامد محمد سيد محمد

١ ش نصر نصيف-كفر الجبل

الهرم - الجيزه

١٩٩٣/١١

٢٩٣ ١١ ١٦ ٢١ ٠٢٦ ٨٥

LC5341099

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

04/28/2026

MDT FORM

Name:	AHLAM Hammed Mohamed //	MRN:	2070
Age:	32 Y	ID:	27001240102062
Menopausal Sta	PRE	Phone:	01507890999
NRS SCORE:		Marital Status:	M
BMI:		Family History:	SISTER OF MOTHER
CO Morbidity:		Reason Of MDT:	NEW DELOFEVED

Clinical Data:

BX:

ER: 8/8 PR: 8/8 HER2: -VE KI67: 25

Sonomammography:

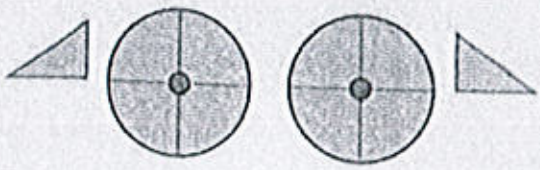
Mets Workup:
NEW CT IN 7/4/2026 REAVELD NEW NOTED RIGHT HILAR AND MED LN

History:
INVASIVE DUCTAL CAR GRADE 2 ON HORMONAL TREATMENY METASTATIC TO BONE AND LN FROM START

Recommendations:

MDT Outcome & Plan
FOR ibrance faslodex with zoladex

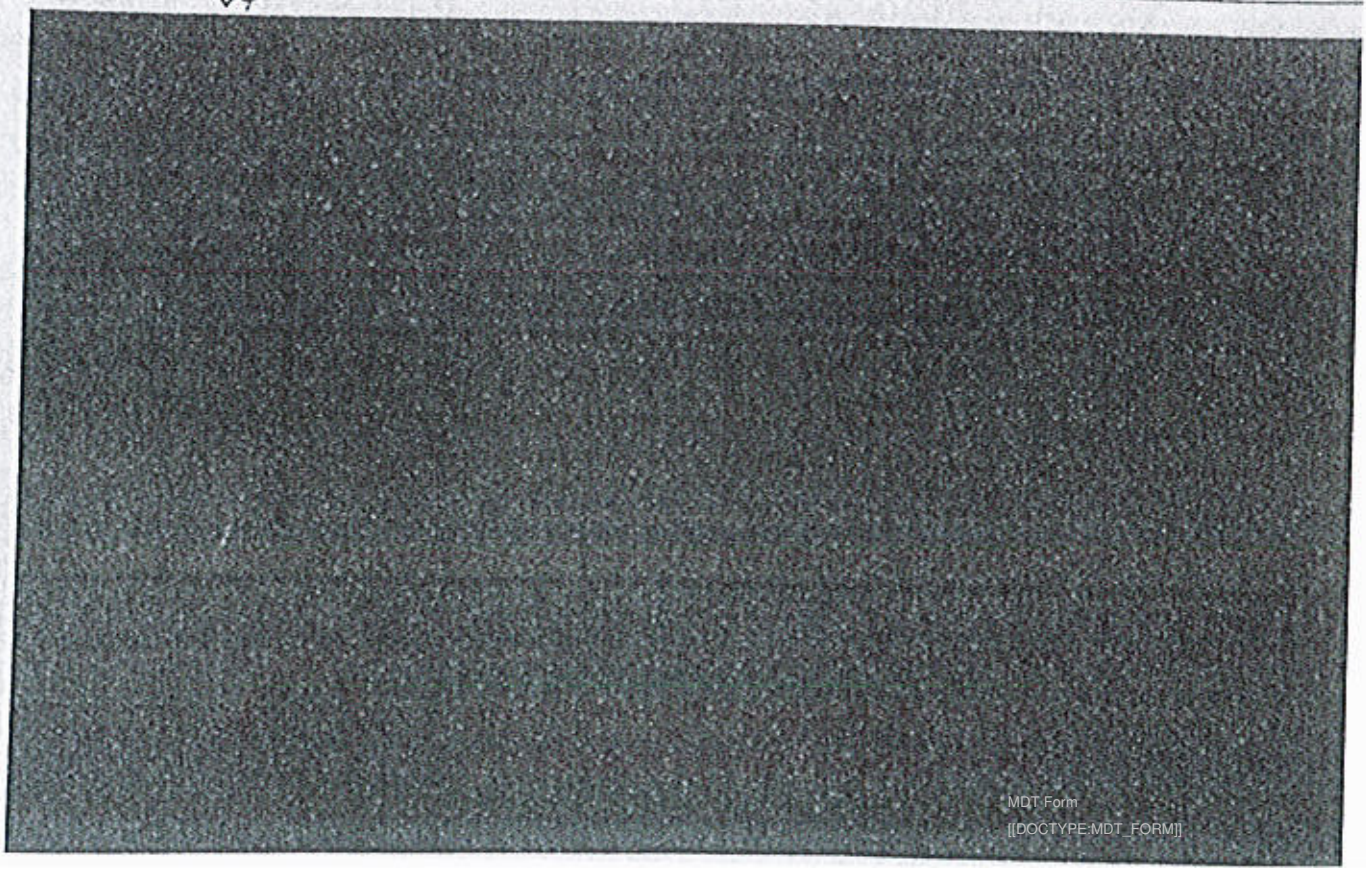
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TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
dr hanan ezzat					



Name: احلام حامد محمد **MR. :** 2070	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with metastatic breast cancer to bone. The attending noted that the patient was seen today for a review of the current management plan, which includes Zoladex 10.8 mg, Arimidex, and supportive therapy. A new CT of the chest was obtained with comparison to prior imaging, showing a right-sided lesion.

The patient's disease progressed to bone metastases in April 2026. The multidisciplinary tumor board recommended Ibrance, Fasoladex, and Zoladex. A CT of the chest in April 2026 revealed newly involved right hilar and mediastinal lymph nodes. The decision was made to administer Zoladex 10.8 mg.

On June 6, 2026, the patient underwent an upper gastrointestinal endoscopy. ALT was noted to be 232 U/L. The patient reported bone pain and vomiting. The plan was to initiate palbociclib and olaparib.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **Reham Ahmed Fathy Ibrahim**

يعتمد ,

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient's Name:	**AHLAM HAMED MOHAMED SAYED**
Date:	**2026-04-07**
Referred by:	Prof.Dr. CT CAP+C
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma , compared to last study dated on 20/8/2025

(I) Chest:

- **Right breast architectural distortion with skin thickening**
- **Previously noted right breast mass ot barely appreciated .**
- **Newly noted Few enlarged right hilar and mediastinal lymph nodes , the largest at right hilar region 17 x 12 mm . .**
- **Bilateral scattered pulmonary nodules , the largest at lingula measures 6mm.**
- **Clear both lung fields .no sizable pulmonary masses or consolidation patches**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**

(II) Abdomen and pelvis:

- **The liver is enlarged in size showing homogenous low CT density , few bilobar lesions , the last seen at segment II measures 2.6**

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient's Name:	**AHLAM HAMED MOHAMED SAYED**
Date:	**2026-04-07**
Referred by:	Prof.Dr. CT CAP+C
Hospital:	**Goustave Roussey International Egypt**

x 2.1 cm compared to 2.5 cm with no appreciable changes

- **Patent and homogenously enhancing portal vein.**
- **No intra or extra hepatic biliary radicles dilatation.**
- **Normal CT appearance of the pancreas, both kidneys, aorta and IVC.**
- **Normal CT appearance of the spleen .**
- **Regular filling of the urinary bladder showing no stones, masses .**
- **Unremarkable other pelvic organs.**
- **No retro-crural, para-aortic or pelvic lymphadenopathy.**
- **No ascites.**
- **No appreciable changes regardingwWidespread sclerotic lesions of axial and appendicular skeleton**
- **No vertebral structural collapse .**

OPINION:.

- **Newly noted Few enlarged right hilar and mediastinal lymph nodes**
- **Previously noted right breast mass not barely appreciated**
- **Rather stationary course rest of the study**

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Ahlam Hamed Mohamed	Date : 25/09/2024
Exam : C.T CHEST	Age : 31 Year(s)
Medical ID: 809/2024	Pat. ID: 49381

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- Reported case of breast cancer.
- ***Compared to last CT study dated 06/07/2024, the current study revealed:***

I Chest:

- Stationary course of the right breast diffuse parenchymal distortion with mild overlying skin thickening.
- Further regression of enlarged ipsilateral discrete and confluent right axillary (level I, II) lymph nodes being currently subcentimeteric.
- Rather stationary coarse of the associated Few prominent ipsilateral right supraclavicular lymph nodes, largest measures about 4 mm compared to 4.5 mm.
- Stable few subcentimetric subpleural pulmonary nodules.
- Prominent retrosternal thymic tissue is seen
- No sizable pulmonary masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is enlarged showing rather stationary course as regards the size of the multiple variable sized hypodense focal lesions, largest at segment III having subcapsular location measuring about 4.1 cm (on non-contrast basis) compared to 4.3 cm in axial dimension.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للمراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Ahlam Hamed Mohamed	Date : 25/09/2024
Exam : C.T CHEST	Age : 31 Year(s)
Medical ID: 809/2024	Pat. ID: 49381

- Normal CT appearance of the spleen, pancreas, kidneys, suprarenals glands, IVC and aorta.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Still seen prominent para-aortic and aorto-caval lymph nodes.
- No ascites.

III The scanned Skeleton:

- Sclerotic transformation of the previously seen multiple lytic osseous lesions involving axial and appendicular skeleton **denoting good response of treatment.**

Opinion:

- **Further regression of right axillary lymph nodes.**
- **Sclerotic transformation of thickening previously reported osseous lesions denoting favorable response.**
- **Rather stationary course of the multiple hepatic focal lesions.**
- **Stationary course of the rest of CT findings.**

Dr. Marina Emad

Signature Prof. Menna Tallah Hanafy

15/10/2024 12:01 م

CT Scan
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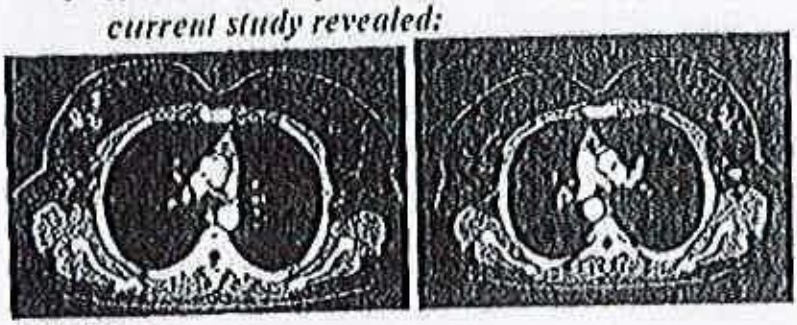
GUSTAVE ROUSSY
INTERNATIONAL
AKOSKY

Patient Name: Ahlam Hamed Mohamed	Date : 20/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 31 Year(s)
Medical ID: 809/2024	Pat. ID: 49381

POST CONTRAST-MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- *Known case of breast cancer, compared to the previous study report dated 25-9-2024, the current study revealed:*



Findings:

- Still noted right breast diffuse parenchymal distortion with mild overlying skin thickening currently reaching 7mm.
- Right breast upper outer quadrant soft tissue lesion measuring about 2x3.3 cm for correlation with the unavailable previous films.
- Still noted right axillary lymph nodes, the largest seen measuring 11.5x9 mm.
- Few subcentimetric left axillary lymph nodes are noted.
- Still noted few subcentimetric subpleural pulmonary nodules.
- Still noted prominent retrosternal thymic tissue.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.
- The lower neck cuts revealed few bilateral thyroid lobes hypodense nodules for ultrasound correlation.

OPINION:

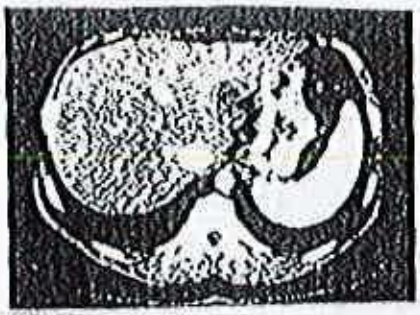
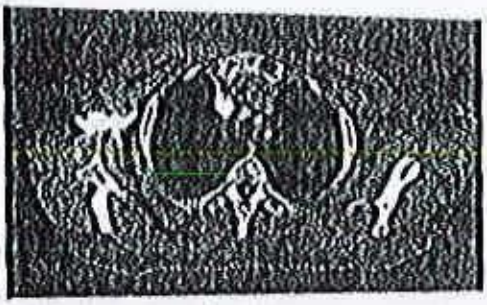
- *Still noted right breast diffuse parenchymal distortion with mild overlying skin thickening.*
- *Right breast upper outer quadrant soft tissue lesion for correlation with the unavailable previous films.*
- *Still noted right axillary lymph nodes.*
- *Few subcentimetric left axillary lymph nodes.*
- *Still noted few subcentimetric subpleural pulmonary nodules.*
- *Still noted prominent retrosternal thymic tissue.*

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS: -

Clinical History:

- *Known case of breast cancer, compared to the previous study report dated 25-9-2024, the current study revealed:*

CT Scan
[[DOCTYPE:CT_SCAN]]

Findings:



- The liver is still enlarged (17 cm) with still noted hypodense focal lesions, the largest still seen at segment III/IV currently measuring about 2.5 cm in diamter (compared to 4.1 cm).
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size of the spleen (9 cm) with no focal lesions seen.
- Normal size, parenchymal thickness and density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Still seen para-aortic and aorto-caval lymph nodes being subcentimetric in the current study.
- Average filling of the urinary bladder with no sizable stones or masses.
- Still noted sclerotic transformation of the previously seen multiple lytic osseous lesions involving axial and appendicular skeleton.
- Scattered axial and appendicular skeleton multiple lytic lesions for correlation with the unavailable previous films.

Opinion:

- *Still noted hepatomegaly with hypodense focal lesions showing regressive course).*
- *Still noted sclerotic transformation of the multiple lytic osseous lesions involving axial and appendicular skeleton.*
- *Scattered axial and appendicular skeleton multiple lytic lesions for correlation with the unavailable previous films.*

Marina Essam, MSc
Dr.Noura Nabil,ME

CT Scan
[[DOCTYPE:CT_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : [illegible] محمد سيد
Sex: Female
3- Medical number: 83194040
Old Med : 809-2024

3-ID number: 29311162103685
Patient Age : 31 Y

MDT

Family History	-ve	Phone	01554930032	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	12/05/2024
Co morbidity	-ve	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :

Mets Workup :
ct cap:rt breast UOQ lesion 1.9 x201 cm
rt axillary LN 1.3 cm
RT SCV ln 7 mm
HFLs largest seg III 3.8 cm
multiple lytic lesions
bone scan: requested

HX	ER	PR	Her 2 Neu	SISH	KI67
IDC gll with lobular features	8/8	8/8	n		35

T	N	M
		LIVER BONE LN

Sonomammography
6/2/2024 bilat SM: rt breast lesion 11-12 oclock 1.6 x 2.1 cm BIRADS 4c
pathological rt axillary LN+bilateral few cysts

PATIENT HISTORY
Pt initially mets to liver , bone and LN.

MDT OUTCOME & PLAN

- Palliative Radiotherapy
-AI., Zoladex
- Zometa monthly.

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	الأستاذه / احلام حامد محمد سيد محمد		
Patient No.	29311162102685	Request No.	13000264
Path No.	755/24	Request Date	27/02/2024
Sex	Female	Verification Date	11/03/2024
Age	30 Y	Financial Category	اورام دار السلام - مبادره
Referreing Doc.		Department	Out Patient
		Registered by	.marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, Intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guldeline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبانى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الآنسة / احلام حامد محمد سيد محمد		
Patient No.	: 29311162102685	Request No.	: 13000264
Path No.	: 755/24	Request Date	: 27/02/2024
Sex	: Female	Verification Date	: 11/03/2024
Age	: 30 Y	Financial Category	: اورام دار السلام - مبادره
Referreing Doc.	:	Department	: Out Patient
		IHC No.	:
		Registered by	: .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right b east mass.
Nature of Specimen: Tru-cut biopsy, collected at 26/2/2024, 10:30 am.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive contro showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with lobular features.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (sco e 0).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
Dr. Sagda El-Kady

CAP accredited
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]